

JoshHorses Field Guide

A practical participant guide to equine-assisted therapy for anxiety, embodiment, boundaries and assertiveness

The one question to carry into the arena

Can I notice what is happening in me, remain present, communicate clearly without forcing, and stay responsive to the horse's response?

Before the session - 60 seconds

- **Orient.** Look around and notice three ordinary things in the environment. Let your eyes move.
- **Body.** Notice breath, jaw, shoulders, chest, belly, hands and whether you can feel your feet on the ground.
- **Impulse.** Is there an urge to retreat, please, freeze, hurry, perform, or control?
- **Choose one intention.**

Assertiveness: clarity without domination

Horse work can make communication patterns visible because another living being responds in real time. The goal is not compliance at any cost.

DISAPPEARING	CLEAR + RESPONSIVE	FORCING
Vague request Over-apologising Giving up immediately Trying to avoid all discomfort	Know what you are asking Use proportionate energy Allow time for a response Notice and adapt Respect both sets of boundaries	Escalating mainly because uncertainty feels intolerable Treating compliance as proof Losing curiosity about the horse

If anxiety rises

- **Where is it?** Locate the sensation physically.
- **What wants to happen?** Run, rush, freeze, appease, control?
- **What is actually happening around me?** Re-orient to the real horse, facilitator and environment.
- **What is one proportionate next action?** A clear request, a pause, more distance, asking for help, or stopping.

After the session - three minutes

- **Body:** The strongest physical signal I noticed was ...
- **Moment:** The moment with the most charge was ...
- **Impulse:** I wanted to ...
- **Action:** I actually ...
- **Meaning:** What I am wondering about is ...
- **Transfer:** One ordinary-life situation where I can practise this is ...

What progress looks like

Look across weeks for earlier recognition of activation, clearer requests, better tolerance of disagreement or uncertainty, less automatic appeasing/withdrawing, and quicker recovery after stress. A useful session does not have to feel calm or profound.

What the evidence says in 2026

The most defensible summary is: **promising, but not definitive**. Equine-assisted interventions are heterogeneous and the research base still contains small samples, inconsistent terminology, variable methods and relatively few strong randomised trials.

2025 systematic review

Kapteijn et al. reviewed 31 eligible studies involving PTSD, anxiety and/or mood disorders. All quantitative studies reported at least one positive treatment effect, but well-designed studies were sparse and methodological heterogeneity was substantial. DOI: 10.5152/pcp.2025.24958

Two anxiety-relevant studies

- **Project Stride (2015):** a tiny randomised pilot (12 young women) combining equine activities with CBT strategies reported larger social-anxiety reductions than a no-treatment control. DOI: 10.1016/j.explore.2015.08.003.
- **Earles, Vernon & Yetz (2015):** 16 participants completed six weekly two-hour horse-based sessions; self-reported PTSD and generalised anxiety fell and mindfulness strategies increased. No randomised control group. DOI: 10.1002/jts.21990.

Keep the sceptical literature too

A 2014 systematic review concluded that the evidence available at that time did not justify broad clinical marketing claims. The literature has grown since then, but newer reviews still identify methodological weaknesses. A good resource base should hold both facts at once.

Claims to hold lightly

- “The horse always mirrors your emotions.”
- “The horse knows what you feel better than you do.”
- “Heart-rate synchrony is the mechanism that causes healing.”
- “Every movement of the horse has a psychological meaning about the participant.”

A firmer conceptual foundation

Experiential learning, behavioural rehearsal, present-moment attention, interoception, practising boundaries and clear requests, tolerating uncertainty, relationship/repair, metaphor and transferring an in-session experience into ordinary life are all more defensible ways to think about possible mechanisms.

Listening, safety and horse welfare

If you have one hour

Recovery Lab #143 - “Healing Without Words with Stephanie Hathorn” (about 54 minutes) is the best single introductory listen in this resource base for a participant. Search the Recovery Lab show in Spotify for Episode #143.

Second listen

Equine Assisted World - “Misery Is Optional: Redefining Softness, Structure & Anxiety” with Mark Rashid and Crissi McDonald. Particularly useful for thinking about pressure, regulation, clarity and force.

Horse welfare is part of ethical practice

A therapy horse is a sentient participant, not equipment. International and professional guidance emphasises monitoring health, behaviour, stress/fatigue, species-appropriate needs, social contact, suitable workloads, competent handling and opportunities for choice/agency.

- Follow the on-site practitioner/equine specialist's horse-safety instructions.
- Wear appropriate closed footwear and never wrap a lead rope around your hand or body.
- Tell the facilitator if you feel physically unsafe, overwhelmed or unsure what to do.
- Do not assume a calm-looking horse cannot react suddenly.
- Do not assume reluctance should always be overcome; pain, fear, confusion, fatigue and preference are all possible contributors to horse behaviour.

Best welfare reference

IAHAIO International Guidelines on Care, Training and Welfare Requirements for Equines in Equine-Assisted Services. Also see PATH International's Five Domains material and the 2024 systematic review by Baxley, Rodriguez-Mori & Anderson (DOI: 10.1016/j.applanim.2024.106424).

Where to go next

The full JoshHorses repository contains the books list, podcasts, systematic reviews, Australian/Dubbo resources, welfare guidance, a printable session worksheet, and a privacy-safe source audit from Gmail/Google Drive.

Repository: <https://github.com/joshualparris/JoshHorses>

Use this guide to become more available to the experience - not to perform therapy correctly.